Chronic Hepatitis — Autoimmune Hepatitis

1. IgG balloon (gold star)

WHAT YOU SEE

Giant golden 'IgG' balloon with a gold star, held by the woman center-top.

WHAT IT ENCODES

GOLD STAR hallmark: hypergammaglobulinemia / elevated IgG is the signature lab of autoimmune hepatitis. Marked polyclonal IgG elevation plus positive autoantibodies in a hepatocellular (high ALT/AST) pattern points to AIH. IgG also tracks treatment response.

2. Inflamed liver handbag

WHAT YOU SEE

Woman holding a red 'inflamed liver-shaped handbag', center.

WHAT IT ENCODES

AIH = chronic immune-mediated hepatocyte injury, classically in women, often with other autoimmune diseases (thyroiditis, celiac, T1DM). Presents from asymptomatic transaminitis to acute/fulminant hepatitis; untreated progresses to cirrhosis.

3. ANA lady (Type I)

WHAT YOU SEE

Older woman wearing an 'ANA' mask/sash, left.

WHAT IT ENCODES

Type I AIH (most common, adults): ANA positive. The classic adult autoantibody pattern combined with high IgG. Responds to immunosuppression.

4. Anti-smooth-muscle man (Type I)

WHAT YOU SEE

Mustached man with an 'anti-smooth muscle' sash, left-center.

WHAT IT ENCODES

Type I AIH: anti-smooth-muscle antibody (ASMA, anti-actin) — pairs with ANA. ANA + ASMA + high IgG in an adult woman is the prototypical Type I profile.

5. Anti-LKM1 / anti-LC1 (Type II)

WHAT YOU SEE

Younger masked figures at far left of the party.

WHAT IT ENCODES

Type II AIH: anti-LKM1 and/or anti-LC1 antibodies — typically YOUNGER patients/children, can be more aggressive. ANA/ASMA usually negative in pure Type II.

6. Interface hepatitis (plasma cells)

WHAT YOU SEE

Easel painting labeled 'interface hepatitis' with plasma cells, right (artist painting it).

WHAT IT ENCODES

Biopsy hallmark: interface hepatitis (piecemeal necrosis at the portal–lobular boundary) with a dense PLASMA-CELL infiltrate and rosetting. Confirms diagnosis and assesses fibrosis/cirrhosis before treatment.

7. Prednisone tray

WHAT YOU SEE

Doctor offering 'prednisone' on a goblet/tray, center-right.

WHAT IT ENCODES

First-line treatment: prednisone (or prednisolone) to induce remission, tapered as ALT/IgG normalize. Budesonide is an alternative for induction in NON-cirrhotic patients (high first-pass metabolism, fewer systemic steroid effects); avoid budesonide if cirrhotic.

8. Azathioprine

WHAT YOU SEE

'azathioprine' labels by the doctor, right-center.

WHAT IT ENCODES

Azathioprine is added as the steroid-sparing maintenance agent (combined with prednisone). Check TPMT activity before starting to avoid severe myelosuppression. Long-term maintenance prevents relapse; without immunosuppression AIH progresses.

9. AMA man crossed out (red X)

WHAT YOU SEE

Tuxedoed man with 'AMA' mask, large red X over him, center-bottom.

WHAT IT ENCODES

Anti-mitochondrial antibody (AMA) points to PRIMARY BILIARY CHOLANGITIS, NOT autoimmune hepatitis. AMA-positivity in a cholestatic picture should redirect you away from AIH (rare overlap syndromes exist but AMA is not the AIH marker).

BOARD TRAP

'AMA-positive supports autoimmune hepatitis' → wrong; AMA is the marker of primary biliary cholangitis (cholestatic), not AIH (ANA/ASMA/anti-LKM1).

10. High alkaline phosphatase man crossed out (red X)

WHAT YOU SEE

Masked man holding 'serum alkaline phosphatase' sign, big red X, right-bottom.

WHAT IT ENCODES

A predominantly elevated alkaline phosphatase = CHOLESTATIC pattern (PBC/PSC), not the hepatocellular (high ALT/AST) pattern of AIH. High ALP should steer you to cholestatic disease, not autoimmune hepatitis.

BOARD TRAP

'High alkaline phosphatase is the key AIH finding' → wrong; high ALP = cholestatic disease (PBC/PSC) — AIH is hepatocellular with high transaminases and IgG.